HACHIKO KIDS

Behavioral data from children's play — designed around their autonomy

A note for Ge Tiffany Wang · March 2026

The research gap we're standing in

Latin America is home to *180 million children under 15* — and virtually none of them appear in the studies that shape how we design technology for children.

Every dataset on children's digital behavior and mental health comes from the Global North. The children co-designing with researchers at Oxford, Stanford, and CMU are not the same children navigating WhatsApp family groups at age 6, sharing tablets with siblings, or living in cities where a child psychologist has a 3–6 month waitlist.

Hachiko Kids exists at that gap.

What we built

A mobile app where children aged 4–12 care for a virtual pet — **Luna** — by making daily choices in behavioral scenarios:

> *"Luna wants to play. Does she play alone or with friends?"*
>
> *"Luna is angry. What does she want to do?"*

The child is never evaluated. Luna never dies. The session lasts 60–90 seconds.

Each choice maps to one of **five behavioral dimensions** grounded in developmental psychology: instruction-following, socialization, prosocial behavior, emotional regulation, and general mood. Parents receive a weekly summary with observable patterns and actionable tips — in behavioral language, never clinical.

A pathway to professional care opens organically: if a pattern persists for 3–4 weeks, the app suggests mentioning it at the next pediatrician visit. *The child activates the clinician — not the other way around.*

> No child data is ever sold. No individual data is shared without explicit parental consent. If the system detects signals of self-harm or abuse, a crisis protocol activates — the data is never stored for analytics.

Why this population is scientifically underrepresented — and why that matters

Your work on datafication and children's data autonomy *("Treat me as your friend, not a number")* documented how children feel reduced to data points by systems designed without their consent. That research was conducted in the UK.

Latin American children face compounded versions of those same dynamics — with almost no academic visibility:

- Digital literacy programs are sparse or nonexistent
- Parental mediation of technology is lower — more unsupervised screen time, earlier onset
- Children in Chile's PIE program interact with clinical systems that collect data on them with zero transparency or participation
- **22.5% of Chilean children** meet criteria for a mental health disorder (Vicente et al.) — with no continuous monitoring between clinical visits

A behavioral dataset generated through autonomous, game-based interaction — where the child is the agent of every data point — would be academically unprecedented in this context.

The prevention thesis

We believe the most powerful thing we can do for adult mental health is intervene at age 6, not age 26.

Early childhood is when regulation patterns, social preferences, and emotional response strategies are forming. A child who learns to name emotions through play, whose parents receive concrete weekly guidance, and whose behavioral patterns are visible enough to prompt early professional consultation — that child enters adolescence with a fundamentally different foundation.

Chile's post-pandemic data shows a **72% increase in pediatric psychiatric emergencies (2019–2023)**. Waitlists are 3–6 months long. The system is reactive by design. Hachiko Kids is a prevention layer that fits inside families' daily routines — not a clinical tool, not a replacement for therapy, but the continuous, low-friction signal that makes early action possible.

Prevention at childhood scale is the highest-leverage intervention available for the adult mental health crisis.

Current status

→ **MVP live:** Android APK deployed, Supabase backend active

→ **Pilot:** Recruiting 20–30 families in Chile (March 2026)

→ **Funding pipeline:** Open Salcobrand (UDD Ventures), CORFO Capital Semilla, Wayra Colombia, ChileMass

→ **Regulatory alignment:** Chile's Ley 21.430 (2022) and Ley TEA 21.545 (2023) create institutional demand for exactly this type of tool

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The team

Edgar Gomero

Operated Chile's PIE program at CEAPSI, coordinating psychologists, physicians, and public schools. Knows the clinical-institutional flow from the inside: how resources move, who makes decisions, what breaks. His purpose: quality education adapted to every child.

José María Muñoz

Data Engineer (Databricks, Azure, Spark) with enterprise-scale experience in sensitive data pipelines across LATAM. Built the MVP with AI assistance. His purpose: technology that genuinely helps people.

Why we're reaching out to you

Your framework of **12 design mechanisms for children's digital autonomy** is the closest academic foundation to what we're building. Your observation that children want to be treated as agents — not as data points — is embedded in every design decision we made.

We believe Hachiko Kids could be a fieldsite for research questions your lab is already asking:

1. How do children in LATAM perceive datafication differently from children in the UK or US?
2. Can a behavioral interface designed around autonomy principles generate higher-quality and more ethically grounded data than passive tracking?
3. What does it look like to design for children's data autonomy in under-resourced contexts — where privacy literacy is lower but stakes are higher?
4. Can early behavioral signals detected through play correlate with validated clinical instruments (SDQ, CBCL)?

We're not asking for endorsement. We're asking if there's a conversation worth having — about mentorship, about research collaboration, or simply about whether what we're building is pointed in the right direction.

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"We read 'Treat me as your friend, not a number in your database.' We are trying to build exactly that — for children who never appear in that kind of study."